This procedure is under review and remains the current endorsed procedure until the review is completed.

PROCEDURE	
Code Blue Medical Emergency Procedure - Inpatient MHS	
Scope (Staff):	All NMHS Mental Health (MH) Staff
Scope (Area):	All NMHS Inpatient Mental Health Services (MHS)

Contents

1. Aim

The aim of the Code Blue Emergency Management Procedure is to minimise mortality and morbidity from acute medical emergencies. This procedure provides direction and guidance for medical emergency teams providing a Code Blue response to patients, staff and visitors in mental health services.

2. Background

Code Blue Procedures are established to effectively manage code blue medical emergencies that occur in Mental Health (MH) inpatient services. All staff are required to have knowledge of Medical Emergency Procedures for their service, which are accessible in all services and include code blue activation criteria, code blue response and action cards.

The Code Blue Emergency Management Procedure is to be read in conjunction with the MH Recognising and Responding to Acute Deterioration (RRAD) Policy and the Emergency Management Procedures for Mental Health Services.

3. Risk

Non-compliance with this procedure / guideline / protocol will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☒	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☒	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☐
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☒	Other	☐

4. Definitions

Aishwarya's Care Call (ACC): This is an escalation process that is available to patients, carers and family members if they are concerned or worried about the care provided in the event of clinical deterioration.

Anaphylaxis: A serious potentially life-threatening allergic reaction that is triggered within seconds or minutes of exposure to an allergen and is rapidly assessed for anaphylaxis especially in the presence of an allergic trigger or a history of allergy.

Acute Anaphylaxis: The Australasian Society of Clinical Immunology and Allergy (ASCIA) defines anaphylaxis as:

- Any acute onset illness with typical skin features (urticarial rash or erythema/flushing, and/or angioedema), PLUS involvement of respiratory and/or cardiovascular and/or persistent severe gastrointestinal symptoms; or
- Any acute onset of hypotension or bronchospasm or upper airway obstruction where anaphylaxis is considered possible, even if typical skin features are not persistent.

Code Blue: A Code Blue is a medical emergency with defined medical criteria that requires an immediate medical response to maintain life. Refer to Code Blue Activation Criteria **(Appendix 1)**.

Focal aware Seizures (FAS): originating within networks limited to one hemisphere that are often characterised by an epileptic "*aura*" or a cluster of signs that precede seizure activity.

Preictal state: Refers to the period immediately prior to the onset of a seizure when individuals experience early warning signs that may include flickering lights, blurry vision or partial visual loss, confusion, nervousness, twitching, loud yell or cry and pacing.

Postictal state: The postictal state is the altered state of consciousness after an epileptic seizure. It usually lasts between five (5) and thirty (30) minutes after a tonic-clonic seizure, but sometimes longer in the case of more severe seizures. The patient may experience headaches, nausea, drowsiness, lethargy, exhaustion or hypertension. Post seizure recovery may also include aggression, confusion, agitation or bizarre behaviour.

Seizure: Originating at some point within, and rapidly engaging, bilaterally distributed networks (Berg et al, 2010).

Seizure Observations: Include assessment of level of consciousness (LoC) during seizure, somatic sensory symptoms (tingling metallic taste; seeing flashing lights), hallucinations, motor activity, lip smacking and repetitive gestures. (PMR38)Berg et al, 2010).

Seizure types: Include tonic-clonic seizures, tonic seizures (impairment of consciousness and stiffening), clonic seizures (jerking with impairment of consciousness), absence seizures, myoclonic seizures (brief shock-like contraction of the limbs), and atonic seizures (brief loss of tone associated with falls).

Status Epilepticus: Is a condition resulting either from the failure of mechanisms responsible for seizure termination or from the initiation of mechanisms, which lead to abnormally prolonged seizures. Status epilepticus can have long-term consequences including alteration of neuronal n Goals of Patient Care (GoPC): The GoPC establishes and documents the most medically appropriate, agreed goals of patient care that apply in the event of the patient's clinical presentation and / or deterioration.

Goals of Patient Care (GoPC) Form: (PMR00) documents, communicates and directs the agreed goals of patient care that will apply in the event of the patient's clinical deterioration.

Goals of Patient Care (GoPC) eForm: (MR00H.1) is completed electronically and documents, communicates and directs the agreed goals of patient care that will apply in the event of the patient's clinical deterioration and is accessible on-line.

Medical Emergency: Medical emergency refers to all cardiac and respiratory arrests and all conditions involving acute changes in any one or more of the following; Airway, Breathing, Circulation, Neurology, Urinary Output and any patient of serious concern not fitting the Code Blue Medical Emergency Activation Criteria (**Appendix 1**).

Medical Emergency Response (MER): The MER is in place to manage medical emergencies that occur in response to clinical deterioration outside the criteria for a Medical Emergency Team (MET) response.

Medical Emergency Team (MET): The MET consists of a designated group of healthcare clinicians who can be assembled quickly to deliver critical care expertise in response to clinical deterioration of a person and initiate appropriate clinical management.

MH Observation and Response Chart (ORC) is a multi-tiered individualised patient observation chart with six (6) physiological parameters that guide medical and nursing staff observations, and responses by triggering a mandatory escalation protocol (PMR 38).death (Trinka et al, 2015).

5. Principles

Systems are in place to ensure the delivery of appropriate and timely care in response to a Code Blue medical emergency.

6. Key Points

- Emergency Management Procedures provide direction for staff within all MH services.
- All clinical areas have standardised medical emergency equipment that is checked and documented by nursing staff every 24 hours.
- All staff must be familiar with the emergency equipment and where it is located in their service and know how to activate a Code Blue in their service **(Appendix 1)**.
- All staff working in MH services complete mandatory training in Emergency Management and Basic Life Support (BLS) **(Appendix 2)** in accordance with the MHPHDS Mandatory Training Policy.
- Clinicians providing emergency assistance must use the iSoBAR clinical handover process when communicating with attending medical staff and members of the medical emergency response team and must comply with the elements identified in the WA DoH Clinical Handover Policy.

- The medical lead of the Code Blue response team takes responsibility for emergent care decisions as deemed appropriate in a medical emergency response.
- The ongoing clinical management of the patient remains the responsibility of the admitting/treating doctor.

7. Emergency Management Equipment

All services have standardised emergency equipment that is readily accessible when a medical emergency is called. The following equipment is mandatory for inpatient services;

- Resus Trolley
- Defibrillator/ with ECG
- Oxygen
- Suction
- Airway Management
- Intubation
- Canula/IV fluid
- Emergency Medication
- Anti-Ligature Kit
- MET/Code Blue Report (PMR000)

Nursing staff are responsible for checking, monitoring and maintaining the emergency equipment. Clearly Identified processes must be in place for services to restock emergency trolleys after use and to replace out of date stock when required. Registers are checked daily and signed. A component of daily checking is to ensure that equipment is clean and suitable for use.

8. Code Blue

Providing a timely response by a skilled team, as soon as any life-threatening deterioration in physiological status is recognised, is the most effective response for averting a cardio-respiratory arrest and preserving life.

- A Code Blue is activated when there is a medical emergency requiring urgent medical assistance (**Appendix 1**).
- A Code Blue response is provided by a trained team of doctors and nurses with Basic Life Support (BLS) Including resuscitation skills **(Appendix 2)** and Advanced Life Support (ALS) **(Appendix 3)**.
- Code Blue teams respond to medical emergencies involving a patient, staff member or visitor within hospital services.
- The Code Blue Process identifies key steps involved in activating an emergency medical response, calling a Code Blue and completing the notification, debriefing, documentation and reporting requirements **(Appendix 4).**

8.1 Code Blue Activation

Code Blue Activation Criteria are identified in the Activation Criteria (**Appendix 1**)

Activate a Code Blue Medical Emergency Response and follow DRSABC Action Plan **(Appendix 2)**

Danger - ensure area is safe for yourself others and patient.

Response - check for response. If no response send for help.

Send for Help - Call 55 (medical emergency) or Triple zero (000) for an ambulance and follow the Emergency Management Procedures for the service

Airway - Open mouth. Clear airway and tilt head with a chin lift, jaw thrust to open airway; suction; insert airway to clear and manage airway as required.

Breathing - Check for breathing, look, listen feel. Monitor respiratory rate and oxygen saturation. If not normal breathing start CPR.

CPR - 30 chest compressions: 2 Breaths Continue **CPR** until responsiveness or normal breathing returns.

Defibrillation: Attach Defibrillation and follow the voice prompts maximising team input to coordinate and manage. Monitor pulse, blood pressure, and IV access. Use ECG monitor if patient is tachycardic, bradycardic or pulse is difficult to palpate. Refer to Advanced Life Support (**Appendix 3**).

Clinicians providing emergency Code Blue response must:

- Respond within agreed timeframes.
- Assess the patient and provide information relevant to their management.
- Undertake appropriate assessment and interventions.
- Have authority to escalate care and make transfer decisions as indicated.
- Seek clinical advice from senior staff when necessary.
- Refer to Goals of Patient Care (GoPC).
- Document events surrounding the actions and interventions that resulted from the Code Blue in the patient's medical record.
- Complete the MET/Code Blue Report (PMR000) **(Appendix 5)**.
- Notify the Consultant with primary responsibility for the patient regarding the Code Blue and outcome.

9. Acute Anaphylaxis

Anaphylaxis is a severe, life threatening, generalised or systematic hypersensitivity reaction. This is characterised by rapidly developing life-threatening airway and/or breathing and/or circulation problems usually associated with skin and mucosal changes. According to the Australasian Society of Clinical Immunology and Allergy (ASCIA) there are many definitions for anaphylaxis within literature. The ASCIA defines anaphylaxis as:

- Any acute onset illness with typical skin features (urticarial rash or erythema/flushing, and/or angioedema), PLUS involvement of respiratory and/or cardiovascular and/or persistent severe gastrointestinal symptoms; or

- Any acute onset of hypotension or bronchospasm or upper airway obstruction where anaphylaxis is considered possible, even if typical skin features are not persistent.

9.1 Acute Anaphylaxis Clinical Care Standard

The goal of the Acute Anaphylaxis Clinical Care Standard is to improve the recognition of anaphylaxis, and the provision of appropriate treatment and follow-up care.

The standard supports the delivery of evidence- based clinical care that recognises the following quality statements;

- a locally approved Anaphylaxis Pathway
- immediate injection of intramuscular adrenaline
- correct patient positioning
- access to a personal adrenaline injector in all healthcare settings
- observation time following anaphylaxis protocol
- discharge management and documentation

Anaphylaxis usually occurs within half an hour of allergen exposure but may take up to two (2) hours to develop. Anaphylaxis can be triggered by any of a broad range of allergens with food, drugs, stinging insects, snake venom and latex identified as possible triggers. Food is the commonest trigger in children and drugs the commonest in adults. Virtually any food or drug can be implicated, but certain foods (nuts) and drugs (muscle relaxants, antibiotics, non-steroidal anti-inflammatory drugs and aspirin) cause most reactions (Australian Food Standard Code; HACCP Food Safety Guidelines). Exposure to a known allergen for the patient supports the diagnosis.

9.2 Clinical Criteria for Anaphylaxis

Anaphylaxis is likely when all of the following three criteria are met:

1. Sudden onset and rapid progression of symptoms.
2. Life-threatening Airway and/or Breathing and/or Circulation problems.
3. Skin and/or mucosal changes (flushing, urticaria, angioedema).
   (Refer to Acute Anaphylaxis Clinical Care Standard)

9.3 Anaphylaxis Pathway

The Anaphylaxis Pathway includes:

- Assessment protocol that outlines the clinical criteria to support prompt diagnosis of anaphylaxis;
- Systems for monitoring patients experiencing allergic reactions;
- Process to ensure clinicians are competent in the anaphylaxis pathway; and
- Process to assess adherence to the anaphylaxis pathway.

If the patient has asthma-like features alone, treat as for asthma

Antihistamines have no role in treating or preventing respiratory or cardiovascular symptoms of anaphylaxis. For treatment of anaphylaxis in mental health services, a Code Blue Medical

Emergency must be triggered, and the Emergency Drug Box adrenaline should be used (MH Medication: Prescribing, Administration and Management for Inpatients Policy).

9.4 Observation Monitoring, Diagnosis and Treatment

- As the diagnosis of anaphylaxis is not always obvious, all those who treat anaphylaxis must use the BLS (DRSABC) Australian resuscitation guide **(Appendix 2)** when providing a response.
- Monitor all patients who have suspected anaphylaxis as soon as possible. (Refer to Anaphylaxis Pathway **(Appendix 6)**.
- Minimum monitoring includes non-invasive blood pressure, pulse oximetry and oxygen.
- Treat life threatening problems as you find them.
- Airway obstruction may occur rapidly in severe anaphylaxis, particularly in patients with angioedema.
- Warning signs are swelling of the tongue and lips, hoarseness and oropharyngeal swelling.
- Treat as a medical emergency and initiate a Code Blue if airway is compromised.
- All patients should be placed in a comfortable position.
- Patients with airway and breathing problems may prefer to sit up as this will make breathing easier.
- Lying flat is helpful for patients with low blood pressure; do not stand the patient.
- Remove the trigger if possible. Stop any drug suspected of causing anaphylaxis.
- Treatment of anaphylaxis to prevent cardio-respiratory arrest and the key steps are described in the Anaphylaxis Pathway **(Appendix 6).**
- The use of IV adrenaline applies only to those trained in the use and titration.

9.5 Discharge Management and Documentation

Patients who have had a suspected anaphylaxis should be treated and then observed in a clinical area with facilities for treating life threatening Airway, Breathing, Circulation (ABC) problems **(Appendix 2)**.

Decision about transfer to a tertiary facility must be made for each patient by the treating clinician.

All information relating to the anaphylaxis must be clearly documented in the medical notes, a Clinical Incident Form (Datix CIMS) is completed and details of the allergy/reaction entered onto Clinical Alerts (MH Management of Clinical Alerts Policy).

Before discharge all patients are provided with the following:

- A treatment plan based on their individual risk include contact with the patient's general practitioner.
- Be given clear instructions to return to hospital if symptoms return.
- Be considered for an adrenaline auto-injector or given replacements and ensured that appropriate training has been given.

Discharge Summary is forwarded to identified GP with consideration for a specialist follow up appointment if indicated.

10. Seizures

Epileptic seizure is a common medical presentation and is defined as transient occurrence of signs and/or symptoms due to abnormal excessive neuronal activity in the brain. There are multiple seizure types and of varying severity depending on where the seizure originates.

10.1 Classification of Seizures

- Seizures are generally classified as either focal or generalised in onset and further classified whether awareness is preserved or impaired.
- Most seizures are brief and self-terminate within one (1) to three (3) minutes without drug treatment.
- Generalised absence seizures (petit mal seizures) are frequent (>daily), brief (<30 seconds) episodes of behavioural arrest without prominent motor features and with immediate recovery of alertness when the seizure ends.
- In the initial phase of stiffening, consciousness is lost for a few seconds, causing falls, often with a forced scream. Jerking of the limbs occurs, lasting for up to 2 minutes. Cyanosis, tongue-biting and urinary incontinence may occur. Post-ictal stertor (noisy breathing sound), confusion and amnesia typically persist for several minutes post seizure.
- Seizures lasting five (5) minutes or more are less likely to self-terminate and require an emergency medical response.

10.2 Seizure Management

In the event of a seizure alert the medical officer and request a medical review of the patient. A staff member remains with the patient and assesses and monitors ABC. **Refer to the** Seizure Management Guidelines **(**Appendix 7**)** and the MH Recognising and Responding to Acute Deterioration Policy for ongoing assessment and management of the patient **(Appendix 7).**

10.3 Medical Emergency Response

Most seizures will terminate spontaneously and be managed with medical and nursing interventions and support for the patient. Repeated or prolonged seizures (referred to as status epilepticus) however must be treated as a medical emergency and in these circumstances a code blue is activated. Refer to Code Blue Activation Criteria **(Appendix 1).**

Usually the MO in consultation with the nurse in attendance will request a Code Blue Activation in the following circumstances;

- Immediately if concerned;
- Seizure lasting more than five (5) minutes;
- Cluster of two (2) or more seizures occurring one after another without recovery between seizures;
- Change in conscious level postictal;
- The patient is not known to have seizures and a concern is identified.
- Code Blue Response processes are followed.

Refer to BLS (**Appendix 2**) and ADLS (**Appendix 3**) and Seizure Management Guidelines (**Appendix 7**).

A MET/Code Blue Report (PMR000) is completed for Code Blue and other medical emergencies. A copy is retained in the patient's Medical Record and the progress notes are updated accordingly. A copy of the MET/Code Blue Report is emailed to the MHPHDS.MortalityandCodeBlueReviews@health.wa.gov.au.

Clinical Alerts are reviewed and updated in accordance with the requirements of the (MH Management of Clinical Alerts Policy).

11. Defusing and Debriefing

- Staff are provided with post incident defusing. Information and resources are available to access from NMHS HealthPoint Critical Incident Response.
- A formal team Debrief can be organised by the Nurse Manager/Clinical Nurse Specialist for staff within 72 hours as required.
- Employee Assistance Program (EAP) contact information is available on the NMHS HealthPoint EAP page and posters and pamphlets are available in staff areas for staff to request individual debriefing and support as required.

12. Compliance, Monitoring and Evaluation

- Code Blue incidents will be recorded on Code Blue/Medical Emergency Report **(**PMR000)
- Code Blue incidents will be recorded on Datix-Cims and investigated in accordance with service requirements for the management of Datix-Cims
- HACs are reported via the Business Intelligence Unit (BIU)
- Leadership Committees receive data reports for their services
- The MHPHDS Safety, Quality and Performance report on audit results to the Mental Health Management Committee and MHPHDS Executive (Link to SQP Audit tools).
- Code Blue/Medical Emergency Mandatory Training compliance data is monitored and reported to MHPHDS Executive

Related internal policies, procedures and guidelines
NMHS Clinical Handover Policy NMHS Emergency Management Policy NMHS Emergency Management Guideline Food Safety Guidelines Snake Sighting and Response Procedure MHPHDS MH Emergency Management Procedures MHPHDS Mandatory Training Policy. MHPHDS MH Recognising and Responding to Acute Deterioration Policy NMHS MH Goals of Patient Care Policy NMHS MH Goals of Patient Care Guidelines NMHS eReferral Policy

NMHS MH Medication: Prescribing, Administration and Management for Inpatients Policy

References
WA DOH MP 0086/18 Recognising and Responding to Acute Deterioration Policy WA DOH Recognising and Responding to Acute Deterioration Guideline OD 0657/16 WA Health Consent to Treatment Policy ACSQHC Acute Anaphylaxis Clinical Care Standard 2021 ACSQHC Standard 8 Recognising and Responding to Acute Deterioration ACSQHC Recognising and responding to deterioration in mental State, Scoping Review 2014 WA DoH Clinical Handover Policy. Regulations and provisions of the Australian Food Standards Code HACCP (Hazard, Analysis, Critical, Control, Points) Guidelines

Useful resources
National Safety and Quality Health Service (NSQHS) 2nd Edition, Standard 8: Recognising and Responding to Acute Deterioration. Clinical Governance, Safety and Quality Policy Framework WA Health's strategy for Advance Care Planning education and awareness raising: For health professionals and the community, Prepared by the Advance Care Planning Team of the End-of-Life Care Program, WA Health 2020. Ministerial Expert Panel Report on AHDs 2019 Advanced Care Planning Australia Education Capability Framework Guide 2020 St John Ambulance, DRABCD Action Plan, www.stjohn.org.au Berg AT, Berkovic SF, Brodie MJ, et al. Revised terminology and concepts for organization of seizures and epilepsies: report of the ILAE Commission on Classification and Terminology. *Epilepsia* 2010; 51:676-685. Trinka E, Cock H, Hesdorffer D, et al. A definition and classification of status epilepticus – report of the ILAE Task Force on Classification of Status Epilepticus. *Epilepsia* 2015; 56:1515. Kapur J, Elm J, Chamberlain JM, Barsan W, Cloyd J Lowenstein D et al. Randomized trial of three anticonvulsant medications for status epilepticus. N Engl J Med. 2019 Nov; 381:2103-13.

Policy Sponsor	MHPHDS Director of Nursing				
Policy Contact	MHPHDS Director of Nursing				
Date First Issued:	13/06/2022	Last Reviewed:	N/A	Review Date:	13/06/2025
Approved by:	MHPHDS Director of Nursing			Date:	21/02/2022
Endorsed by:	MHPHDS Director of Clinical Services			Date:	13/06/2022
NSQHS Standards Applicable:	☒ Std 1: Clinical Governance ☐ Std 2: Partnering with Consumers ☐ Std 3: Preventing and Controlling Healthcare Associated Infection ☐ Std 4: Medication Safety			☐ Std 5: Comprehensive Care ☐ Std 6: Communicating for Safety ☐ Std 7: Blood Management ☒ Std 8: Recognising and Responding to Acute Deterioration	
National Standards for Mental Health Services	☐ Std 1: Rights and Responsibilities ☐ Std 2: Safety ☐ Std 3: Consumer and Carer Participation ☐ Std 4: Diversity Responsibility ☐ Std 5: Promotion and Prevention ☐ Std 6: Consumers ☐ Std 7: Carers ☐ Std 8: Governance, leadership and management			☐ Std 9: Integration ☐ Std 10: Delivery of Care ☐ 10.1 Supporting Recovery ☐ 10.2 Access ☐ 10.3 Entry ☒ 10.4 Assessment and Review ☐ 10.5 Treatment and Support ☐ 10.6 Exit and Re-entry	
Chief Psychiatrist's Standards for Clinical Care:	Physical Health Care of Mental Health Consumers,				
Printed or personally saved electronic copies of this document are considered uncontrolled					

Document Version Control			
Date	**Version**	**Author**	**Notes**
13/06/2022	1.0	Nurse Director MHPHDS	Initial endorsement; This is a new procedure providing direction and guidance for medical emergency teams providing a Code Blue response to patients, staff and visitors in mental health services.

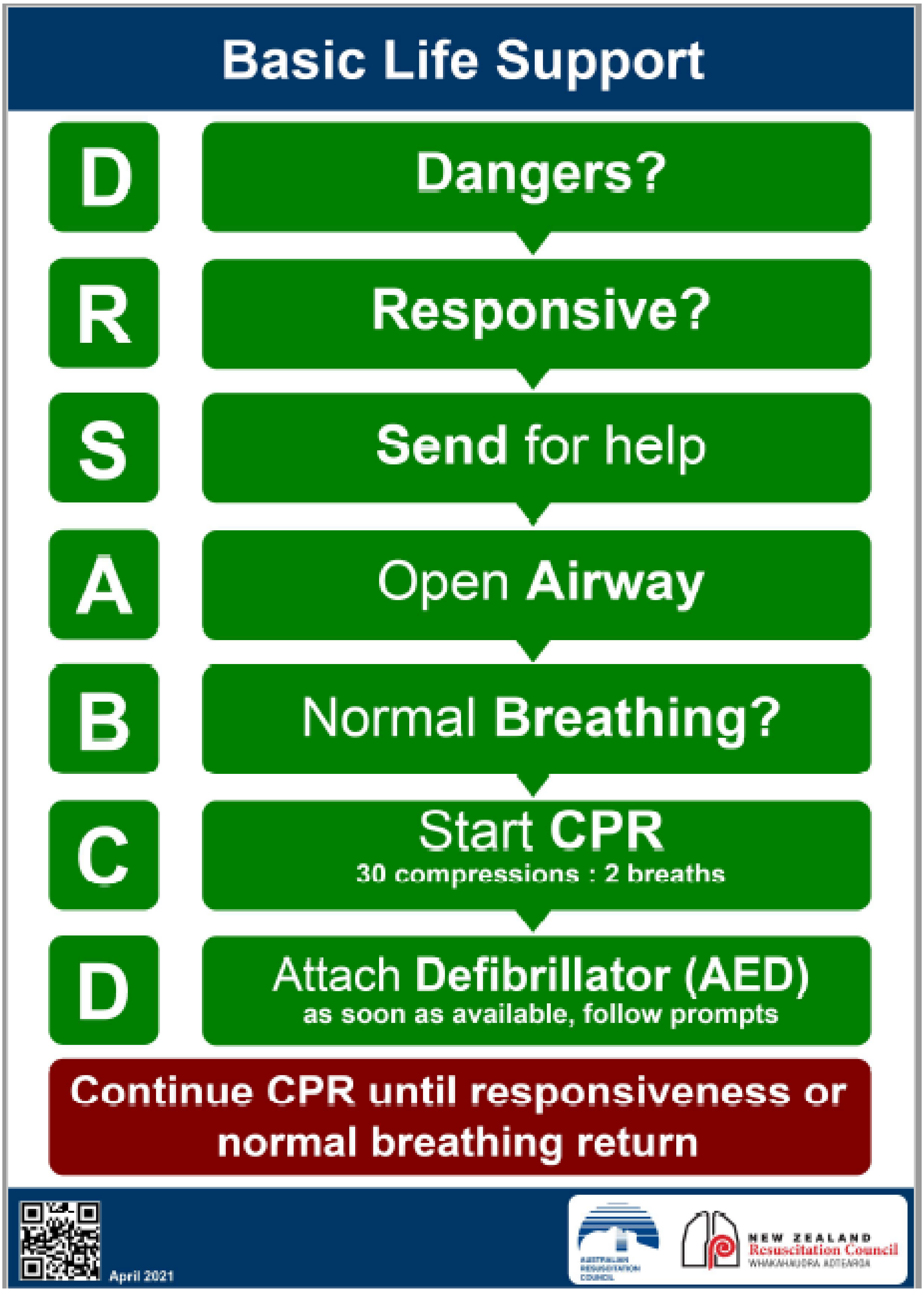
The health impact upon Aboriginal people have been considered, and where relevant incorporated and appropriately addressed in the development of this health initiative (ISD Number 788).

This document can be made available in alternative formats on request for a person with a disability.

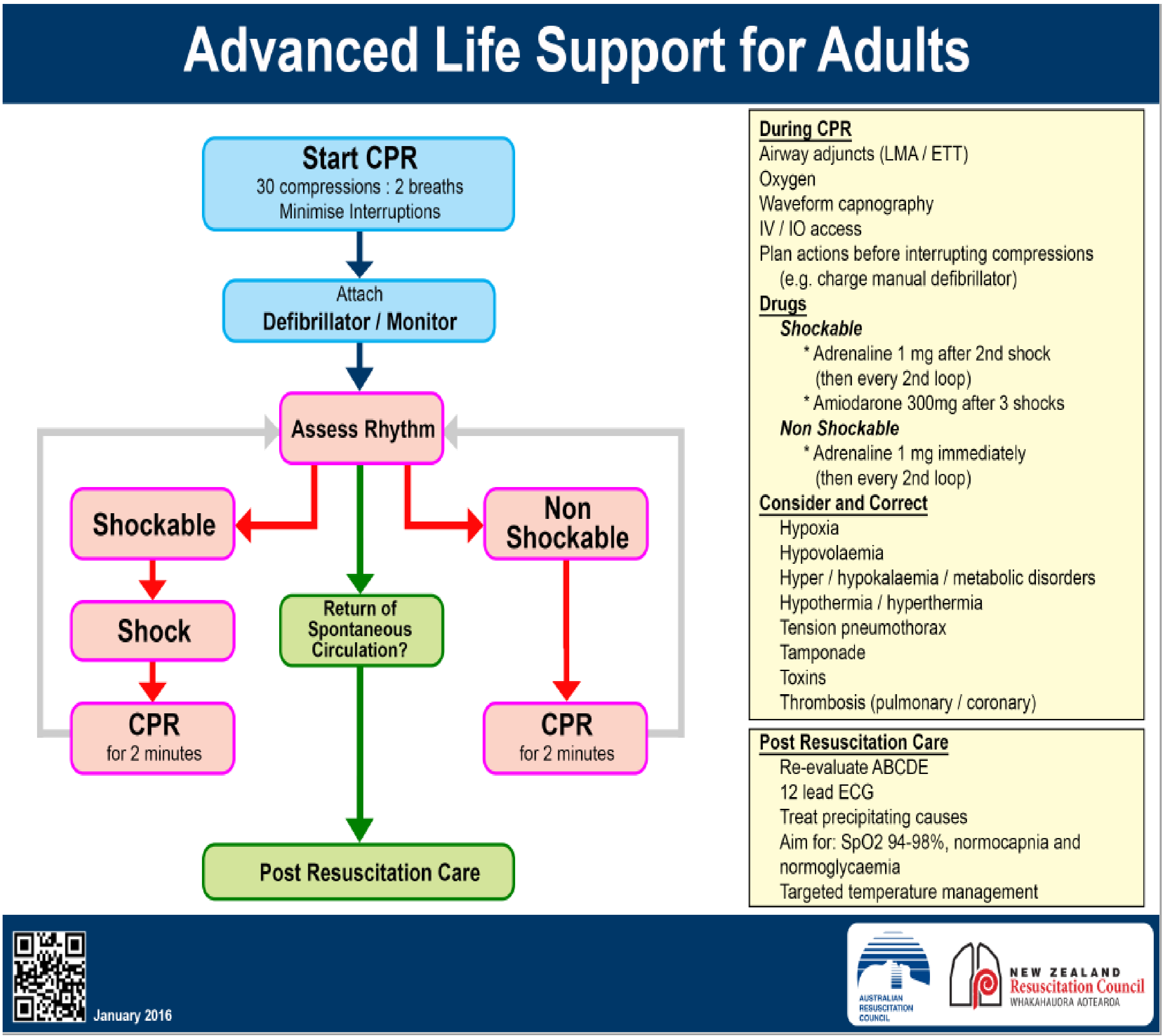
Appendix 1: Code Blue Activation Criteria

CODE BLUE MEDICAL EMERGENCY ACTIVATION CRITERIA All cardiac and respiratory arrests and all conditions listed below	
Acute changes in any one or more of the following	**PHYSIOLOGY**
AIRWAY	• Threatened
BREATHING	• Respiratory Rate ≤4 • Respiratory Rate ≥36 • Oxygen Saturation ≤ 84%
CIRCULATION	• Pulse rate ≤30 • Pulse Rate ≥140 • Systolic BP < 90
NEUROLOGY	• Sudden fall in level of consciousness(>2 points in Glasgow Coma Scale) • Seizure – repeated or prolonged seizures
OTHER	• Any individual who you (or a family member / carer) are seriously worried about who does not fit the above criteria
DIAL '55' and state CODE BLUE – Medical Emergency, the location, your name and position	

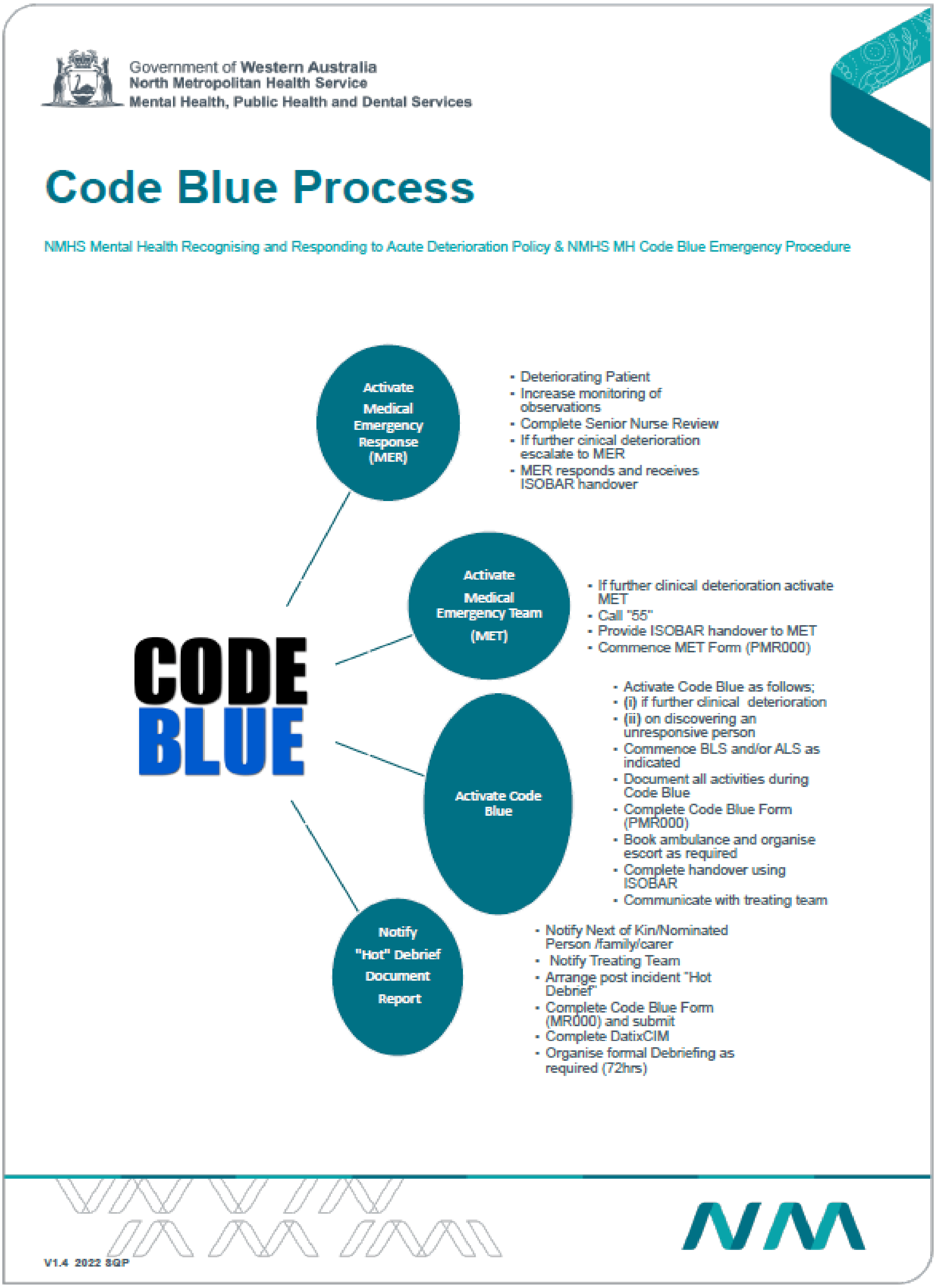
Appendix 2 – BLS Basic Life Support Flow Chart



Appendix 3 – Anzor Adult Cardiorespiratory Arrest Flow Chart (ALS)



Appendix 4 – Code Blue Process



Appendix 5 – MH MET/CODE BLUE REPORT PMR000

NORTH METROPOLITAN HEALTH SERVICE
MENTAL HEALTH

.... Health Service

MET / CODE BLUE

REPORT

Circle for MET or Code Blue

Surname		Sex	U.R. No
Forenames			D.O.B.
Address			
Ward	Registrar	Consultant	
Use Patient I.D. Label when available			

Inpatient Mental Health sites MUST use this MET/Code Blue. A copy should go with the consumer if transferred for medical treatment. Co-located sites use attending site MET/Code Blue Form.

Identifying Details	Situation	Observation Time:________
Date	☐ Pt collapse	*List observations of vital signs at time of call*
Time MET/Code Blue called	☐ Pt self-harm	Resps
Time MET/Code Blue arrived	☐ Airway compromised	O_2 Sats
Location	☐ Repeated or prolonged seizure	Pulse
Name of Person calling MET/Code Blue	☐ Escalation of deteriorating patient	BP
	☐ Worried or concerned ☐ Ambulance Called	Temp
Designation		Consciousness (tick one): ☐ Alert ☐ Responds to Voice ☐ Responds to Pain ☐ Unresponsive
Other Clinical staff assisting		BGL: @ hrs

Background	Actions	Recommendations & Receiver's Acknowledgement
Medical History: (AHD / Goals of Patient Care)	**Time CPR started**	Patient's **Carer/NOK** notified by:
	CPR Initiated by: (Print Name & Designation)	(Print Name & Designation)
	First Defibrillation Shock: hrs No Shock Advised: hrs	on___/___/___@ hrs
	IV Access @ hrs	Name of Receiving Clinician responsible for ongoing Care: (Print Name & Designation)
	Intubation @ hrs	
	Intubation by: (Print Name & Designation)	Receiving Clinician acknowledged care required: (Signature)
Allergies: ☐ Y ☐ N	**Transferred to ED:** @ hrs ☐ MO Letter ☐ Medications List	Incident and ongoing care recorded in Patient Record & Care Plan: (Signature)
Drug:__________	Patient's treating Dr________	Coroner Advised (if applicable)
Reaction:__________	Informed at hrs	on___/___/___@ hrs

RECORDING VITAL SIGNS **LOC- *Alert*, Voice, Pain, Unresponsive (AVPU)**

Time	Resps	O² Sats	Pulse	BP	Temp	A V P U	Rhythm	Defib	Medications	Dose/Route	Initial

MET/CODE BLUE REPORT

PMR 000

Appendix 5 – MH MET/CODE BLUE REPORT PMR000 (page2)

NORTH METROPOLITAN HEALTH SERVICE
MENTAL HEALTH

______________ Health Service

MET / CODE BLUE REPORT

Surname	Sex	U.R. No
Forenames		D.O.B.
Address		
Ward	Registrar	Consultant
Use Patient I.D. Label when available		

Time	Comments – Each entry must have time and initials	Initials

Medications	Dose	Frequency	Time last administered

MET/Code Blue Attending MO Print Name ______________ Signature ______________

MET/Code Blue Nurse Documenting Print Name ______________ Signature ______________

SECTION 1: MET/Code Blue Nurse (or delegate) COMPLETE TABLE BELOW ONCE TASKS ARE COMPLETED

☐ Yes	Place '**ORIGINAL**' **Code Blue report** in patient's Medical Record and a 'copy' to go with patient if transferred
☐ Yes	Forward copy of this form **plus copy ORC** to Coordinator of Nursing within **24 hours** of Code Blue
	DATIX CIMS Reference Number

SECTION 2: Completed by Coordinator of Nursing (CoN) or delegate

DATIX CIMS Reference Number documented ☐ **Yes**

Reviewed by CoN: ☐ Yes__________Date **Reviewed by Head of Service (within 7 days)**__________**Date**

CoN to table at Service **Leadership Committee** ☐ **Yes**__________**Date** **Sign**

Actions/Recommendations	By Whom	By When

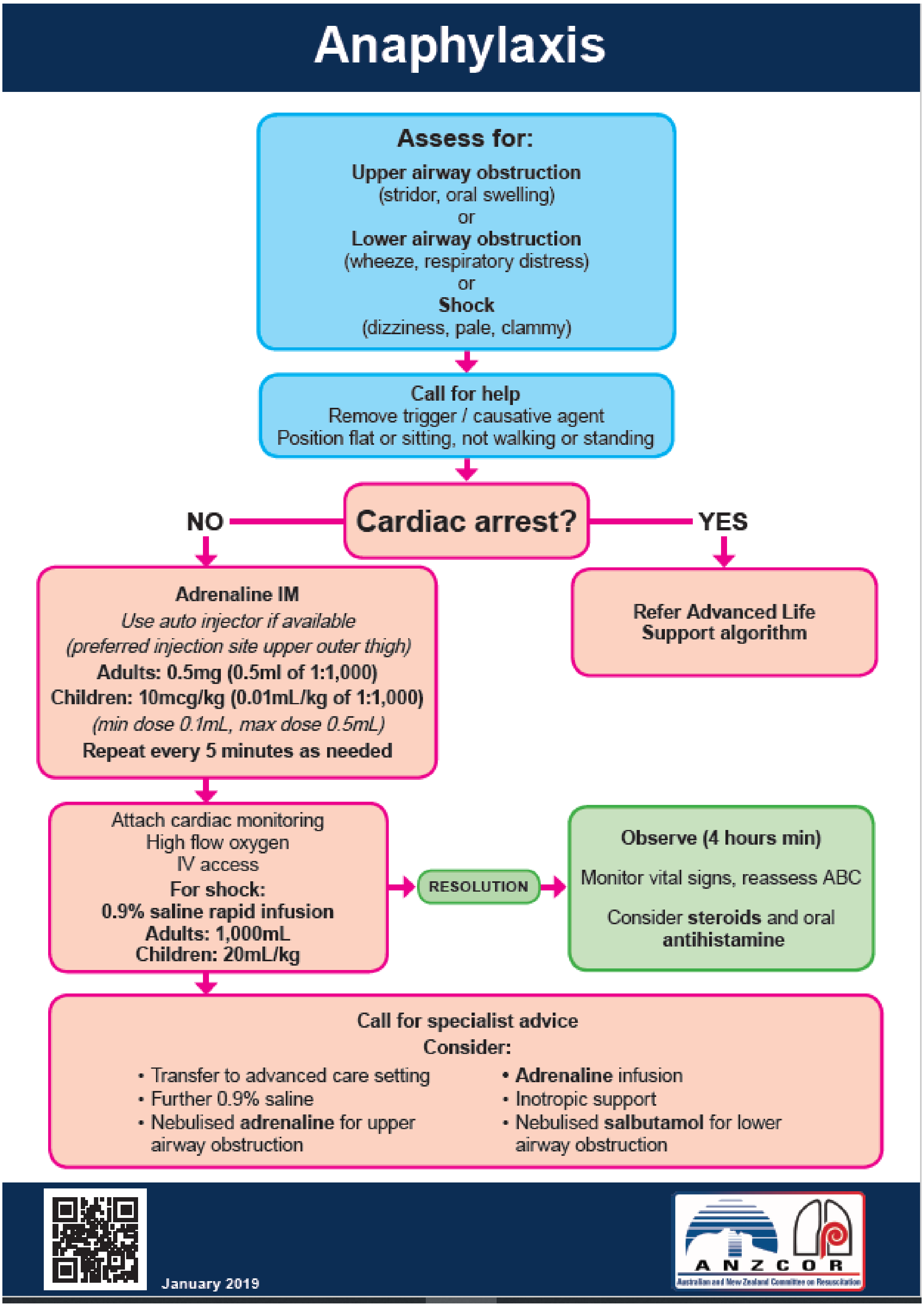
CoN provides feedback to Clinical Team ☐ **Yes** ________ **Date** ____________ **Sign**

CoN submits completed MET/Code Blue **PMR000** Form **plus** copy of **ORC** to:

MHPHDS.MortalityandCodeBlueReviews@health.wa.gov.au

SQ&P presents Code Blue data at **MHPHDS Clinical Governance Committee**

Appendix 6 – Anaphylaxis Pathway



Appendix 7 – MH Seizure Management Guidelines

Initial Assessment	**Protect patient from harm.**
Airway	Ensure clear airway; gentle suction if required
Breathing	Record respiratory rate
	Auscultate chest
	Monitor oxygen saturation
Circulation	Check pulse (cardiac arrest may present as seizure)
	Record blood pressure
	Monitor ECG
Disability	Assess conscious level - AVPU
	Check BGL (hypoglycaemia can cause seizures)
Exposure	Record type and duration of seizure activity
IF AIRWAY, BREATHING OR CIRCULATION COMPROMISED - activate CODE BLUE / MET call	
PROLONGED (> 5 mins) OR REPEATED SEIZURES – activate CODE BLUE / MET call	
ABC stable	**Urgent MO review**

Airway	Perform airway manoeuvres as required Administer **Oxygen 10 Litres/minute via a Hudson Mask**
Breathing	Support breathing with bag and mask device as required
Circulation	Obtain IV access & send blood for analysis (FBC, U&Es, LFTs)
	If BGL <3.5mmol/L give 50mL IV Glucose 50% (or 1mg IM Glucagon)
Seizure > 5mins	Administer **Midazolam 5mg IV *(10mg IM if IV not available)*** **2.5mg IV Midazolam advised for patients >65yo or weight <40kg**
Seizure >10mins	Repeat above dose **IV Midazolam 2.5-5mg** - total max 10mg *(20mg if IM)*
Consider	**Levetiracetam 30mg/kg** (max 4500mg/dose)
OR	**Sodium Valproate 30mg/kg** IV (max 3000mg/dose)
	Phenytoin 20mg/kg IV (max 2000mg/dose)

Seizure activity continues for 30 minutes	
Initiate second anticonvulsant immediately (refer above)	
MO to assess if patient should be intubated and referred to ED for further management	
Continuing Care post seizure activity	
Patient at baseline	Closely observe patient for minimum of 4 hours
Documentation	Document seizure activity and treatment in patient notes
Follow-up: Consult Neurologist and consider:	Optimisation of anti-epileptic medication Investigations to exclude reversible causes

Adapted from FSFHG Adult Seizure Management Clinical Guideline, 2019

Appendix 8 – MH Epileptic Seizure Management Chart PMR46

NORTH METROPOLITAN AREA HEALTH SERVICE
MENTAL HEALTH

.... Hospital

EPILEPTIC SEIZURE MONITORING CHART

Surname		Sex	U.R. No
Forenames			D.O.B.
Address			
Ward	Registrar	Consultant	

Use Patient I.D. Label when available

MENTAL & PHYSICAL STATE POST SEIZURE, RECOVERY & COMMENTS	
TREATMENT, MEDICATIONS ADMINISTERED	
DURATION OF SEIZURE	
SEIZURE OBSERVATIONS	
MENTAL & PHYSICAL STATE TO SEIZURE	
DATE / TIME	

G060
10/09

PMR 46 EPILEPTIC SEIZURE MONITORING CHART